

Q What will happen at my first appointment with the doctor?

The first appointment is a chance to identify your needs (such as additional care for preexisting conditions, extra monitoring for risk factors, or support for personal circumstances), discuss your options for screening tests, and do a health checkup. You and your doctor have a lot to discuss, so allow a couple of hours for the first appointment. He or she will:

» Discuss your general medical history.

Give your doctor details of any illnesses or surgeries you've had, and about any medications you're taking.

» Ask you questions to get to know you, your circumstances, and your family history.

» Take a history of your gynecological health, including your menstrual cycle, use of birth control, and details of any previous pregnancies (including your labors and your babies' health and weight at birth.)

» Ask about your pregnancy symptoms (fatigue, nausea, and so on) and whether you've had any bleeding.

» Ask about how you're feeling emotionally.

It's normal to feel anxious—even terrified and confused!—so don't be worried about saying so.

» Do a basic health checkup to check for general well-being, including taking your BMI (see p.51) and blood pressure, and a urine test to check for infections. Your doctor may take blood samples if you opt for screening tests.

» Advise you about optional screening tests for your baby, such as screening for spina bifida and Down syndrome (see pp.95-9).

» Give you information about nutrition and diet, exercises (including Kegel exercises), your baby's development, childbirth classes, planning your labor and place of birth, and breast-feeding.



You can ask your doctor any **pregnancy-relation question**, at this appointment, from diet concerns to questions about having sex.



How often will I have follow-up appointments?

In a straightforward pregnancy, you will have your regular prenatal appointments at 8, 12, 16, 20, 24, 28, 30, 32, 34, 36, 37, 38, 39, and 40 weeks. If you opt for them, there will also be two ultrasounds—one at 11 to 14 weeks, and another at 18 to 22 weeks. (Your doctor may also offer you an ultrasound at your initial appointment at 8 weeks.) Whether this is your first, second, or third pregnancy, your doctor will see you the same number of times and offer ultrasounds on the same schedule.

Every prenatal appointment will involve a urine test and a blood pressure check. Your doctor will measure your belly (from 24 weeks) to check your baby's growth, and also feel your tummy to establish the position of your baby (from 36 weeks).

You'll be able to talk through any ultrasound or screening test results and implications with your doctor where necessary. Over the course of your appointments, your doctor will discuss with you such topics as labor expectations, the importance of flu shots while you're pregnant, postpartum depression, and breast-feeding.

Q I told my doctor that I was pregnant, and he did tests to confirm this.

Even if you've had a positive home pregnancy test, your doctor may want to confirm your test results with a urine or blood test in the office when you come in for the first appointment. This may simply be standard procedure in your doctor's office, or they may need to measure levels of hormones in your urine or blood when you come in for the appointment. Finding out you have conceived, whether it's planned, unexpected, long-awaited, or quick, can feel instantly life-changing for you, and the extra confirmation you get at your doctor's office may be thrilling.

Q I have a nine-month-old baby and I'm pregnant again. Are there risks?

It's important that you see your doctor as soon as you know you are pregnant because your body can take at least a year to recover before safely undertaking pregnancy again. Problems are, of course, relatively rare, but it's important that you are carefully monitored from the start and advised about how best to manage with both the demands on your body and taking care of a small baby during the tiring early stages of pregnancy. Pregnancies that occur in quick succession raise the risk of placental abruption (see p.147), low birthweight for your baby, and your own nutritional deficiency.

Q My doctor has said that our relationship is confidential. Can I really trust her?

It's important that you and your doctor develop a mutually trusting relationship that enables you to get the appropriate advice and care. You can feel confident that almost anything you could tell your doctor will not be shared with anyone else. Doctor-patient confidentiality requires that your doctor keeps your health information private, unless there's a concern for someone else's safety, including your own. If you have a sexually transmitted disease, a dependency on drugs or alcohol, or you feel especially anxious or depressed, it should remain confidential.